SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Adjuvant ribociclib + endocrine therapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant · HR-positive / HER2-negative early breast cancer · NCCP 00892 · version 2 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
For eligible higher-risk HR-positive/HER2-negative early breast cancer, ribociclib is added to endocrine therapy to reduce recurrence risk.

Schedule
Ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to 3 years. The aromatase inhibitor continues daily on its own schedule; ovarian suppression is added when indicated.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Ribociclib
CDK4/6 INHIBITOR

- ☐ Low white cells and infection
- ☐ Liver injury / abnormal blood tests
- ☐ QT prolongation / heart-rhythm problems
- ☐ Nausea, fatigue and hair thinning
- ☐ Rare serious lung or skin reactions

Non-steroidal aromatase inhibitor ± ovarian suppression
ENDOCRINE PARTNER

- ☐ Hot flushes, joint pain and fatigue
- ☐ Bone loss with AI / ovarian suppression
- ☐ Injection-site discomfort with fulvestrant
- ☐ Drug-specific liver and other effects

Discuss the active phase, supportive medicines, fertility / menopause, treatment alternatives, monitoring, urgent contacts and when the plan will be reviewed.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00892 v2.

SACTCheck v0.83.8
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